

Patient 23**Report A**

657.3853 22/07/2016 MR Head

6573853 22/07/2016 MR MRA

Indication: Patient has right frontal bleed with odema no vascular abnormality? metastatic deposit please MRI to further characterise Findings: Comparison is made to the previous CT imaging 22/07/2016

There is a large right frontal intraparenchymal haemorrhage again noted, with multiple microbleeds and several larger bleeds scattered throughout both cerebral hemispheres, brainstem and left cerebellar hemisphere. There is extensive deep white matter high signal foci on T2/FLAIR in keeping with moderate/severe chronic small vessel disease, especially given the patient's age. The intracranial arteries are unremarkable with no arterial aneurysm, stenosis or occlusion demonstrated.

Conclusion: Large right frontal intraparenchymal haemorrhage with multiple bilateral scattered microbleeds. The appearance suggests underlying aetiology of multiple cavernomata, other causes of a small vessel vasculopathy such as radiation or vasculitis process may be considered, or less likely multiple small haemorrhagic malignant deposits remain within the differential.

Interval scanning at 6-12 weeks (once haematoma has receded) recommended.

Report B

6573261 21/07/2016 CT Angio (Head)

Indication:

Right frontal bleed needs angio for ? vascular aneurysm

Findings:

Note is made of the previous (external) CT head study dated 20/07/16.

The intraparenchymal right frontal haematoma is again noted. Calcified plaques within the right carotid bulb and the cavernous and supraclinoid segments of the ICA bilaterally do not cause significant stenosis.

The cranio-cervical arteries otherwise enhance normally, with no vascular malformation or aneurysmal dilatation demonstrated. DN/PSR